



Gavin Newsom, Governor
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Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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March 27, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Kern Medical Center
Block Transfer Filing No. 2026-HOSP-283
Filed on March 26 2026

Dear Ms. Choate:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Kern Medical Center** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **April 25, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. The Enrollee Transfer Notice (ETN) is missing from the Block Transfer submission. Please provide the ETN for Department review.

Plan Response1: The Plan apologizes for the oversight. Please refer to letter attached below:



Kern%20Medical%20
Center%20Post%20T

2. Please confirm the Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

Plan Response2: The Plan will not transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

3. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Plan Response3: The Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

4. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Plan Response4: The hospital is NOT part of a health system termination.

5. Please confirm whether the contract termination between the Plan and the Terminating Hospital impacts Point of Service (POS) enrollees.

Plan Response5: The contract termination between the Plan and the Terminating Provider impacts Point of Service (POS) enrollees.

6. The Plan indicates that there are zero HMO enrollees impacted. However, the Plan also reports 10 HMO bed days across 4 admissions. Please clarify how HMO bed days and admissions are reported when there are no impacted enrollees.

Plan Response6: Impacted HMO enrollees are those assigned to a provider group that primarily admits to the Terminating Hospital (or that meets the Plan's high-admitter threshold of ≥ 30 admissions in the most recent completed calendar year). No provider groups met these criteria for the Terminating Hospital; therefore, 0 impacted HMO enrollees are reported. By contrast, non-ER HMO admissions and bed days reflect total inpatient utilization at the Terminating Hospital during the most recent completed calendar year (2025), regardless of whether the enrollee's assigned provider group primarily admits to that hospital. Accordingly, the reported 4 admissions totaling 10 bed days reflect utilization by enrollees assigned to provider groups that do not primarily admit to the Terminating Hospital and did not meet the high-admitter threshold.

7. Please confirm if the Terminating Hospital is or provides any of the following:
 - a. Tertiary care, quaternary care, cancer, burn or other high-level specialty services,
 - b. Transplant services, and if so,

- i. What types of transplants,
 - ii. Whether the contract termination includes the transplant contract,
 - iii. The termination date of the transplant contract if separate from the hospital contract.
- c. Recognized or designated as a hospital that provides other high-level specialty services, such as a Center of Excellence for Bariatric Surgery, Blue Distinction Center for Spine Surgery, etc., and if so,
 - i. Identify the distinguished services.

Plan Response7: The Terminating Hospital is designated Trauma Level II.

8. The Plan's PPO line of business is licensed by the Department. Please confirm the following:

- a. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15-mile radius of the Terminating Hospital.

Plan Response8: The Plan will send Department-approved Enrollee Transfer Notices to all PPO enrollees residing within a 15-mile radius of the Terminating Hospital.

9. For each Alternate Hospital, please summarize or confirm the following:

- a. The Plan contacted the hospital regarding the Block Transfer,
- b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer

Plan Response9: The Plan outreaches to the hospitals with occupancy rates at 80% or higher and documented any concerns and capacity response(s) in the filing portal. For hospitals with occupancy rates <80%, justification is not provided as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees per §1300.67.1.3(2)(L): *"Based upon the data made public on the Office of Statewide Health Planning and Development's website, for each of the proposed Alternate Hospitals, provide the available bed occupancy rate for the most recently completed calendar year. If the rate is at 80% or higher, please provide justification as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees."* All alternate hospitals are <80% capacity.

10. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals:

- a. Any out-patient or in-patient services,
- b. Any tertiary care, quaternary care, cancer, burn or other high-level specialty services,
- c. Any transplant services.

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

Plan Response10: The Plan does not anticipate any issues with the alternate hospitals' ability to provide the same outpatient and/or inpatient services available at the Terminating Hospital. The primary distinction is that the Terminating Hospital is designated as a Level II Trauma provider, while the Alternate Hospitals are not. Trauma services constitute Emergency Services. EMS/911-routed adult trauma care, are processed at the in-network benefit level regardless of the contracted status of the receiving facility (including the Terminating Hospital post termination) as members do not select the destination and EMS transports to the nearest appropriate facility based on clinical need and established EMS routing protocols. Accordingly, enrollees requiring emergent adult trauma services following contract termination will continue to receive such services through EMS routing to the nearest appropriate trauma-designated facility, with claims processed at in-network benefit levels.

11. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Plan Response11: To date, there are no enrollees who have a procedure scheduled and authorized by the Plan at the Terminating post-termination.

12. On the Block Transfer Filing form, when asked, "Is the Terminating Provider Group part of a "narrow network" product offered by the Plan?", the Plan answered, "yes" and listed Pathway EPO Exchange Network. The Plan indicated Pathway EPO Exchange Network has zero enrollees in the product. Please confirm there are zero narrow network enrollees with Pathway EPO Exchange Network who are affected by this Block Transfer.

Plan Response12: There are 175 Pathway EPO members residing within a 15-mile radius of the terminating hospital. As with the PPO product, EPO Exchange members may receive care from any provider participating in the EPO Exchange Network. However, unlike the PPO, the EPO does not include out-of-network benefits—members must use in-network providers for coverage (except in applicable emergency situations).

13. Please affirm that the Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

[Plan Response 13](#): The Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

Please submit the Plan's response by **April 3, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in black ink, appearing to read "Stacie Litovchenko". The signature is fluid and cursive, with the first name "Stacie" being more prominent than the last name "Litovchenko".

Stacie Litovchenko
Analyst II
Office of Plan Monitoring